



KMCT MEDICAL COLLEGE HOSPITAL

DEPARTMENT OF RADIOLOGY & IMAGING SERVICES



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NAME	NALINI	AGE / SEX	68Years/F
UHID NO	2024761859	DATE	29-Dec-2025

CT PLAIN & CONTRAST OF ABDOMEN

Technique: Plain & contrast CT abdomen done in multislice CT with 5mm contiguous axial sections with oral, rectal and IV contrast.

- An ill-defined, hypoenhancing solid lesion measuring approximately $3.5 \times 3.6 \times 2.4$ cm (AP $\times$ TR $\times$ CC) is seen in the tail of the pancreas, associated with surrounding fat stranding (desmoplastic reaction). The lesion demonstrates invasion of the splenic hilum, distal one-third of the splenic artery. There is infiltration of the posterior wall of the gastric fundus, evidenced by asymmetric mural thickening (maximum wall thickness ~ 11.6 mm). Suspicious extension into the transverse mesocolon at the level of the splenic flexure is noted, along with direct involvement of the left adrenal gland, which appears bulky and shows heterogeneous enhancement. No dilatation of the main pancreatic duct. Remaining pancreatic parenchyma appears unremarkable.
- The liver is normal in size. Areas of perfusion heterogeneity noted, predominantly in right lobe. Multiple peripherally enhancing lesions are identified, the largest measuring 4.8×2.8 cm in segment VII at a subcapsular location. Another lesion of similar morphology measuring 4.1×3.1 cm is noted in segment VI, demonstrating extracapsular extension with adjacent fat stranding and peritoneal thickening, in keeping with metastatic deposits. The intrahepatic biliary radicles and common bile duct are normal.
- Gall bladder is normally distended. Wall thickness is normal. No calculi/sludge seen. Pericholecystic area appears normal.
- Spleen is normal in size. Few wedge shaped hypoenhancing areas noted, largest measuring 2.4×1.8 cm – infarct likely. Minimal perisplenic fat stranding and fluid noted with adjacent peritoneal thickening. Distal two third of splenic vein not visualized - likely chronically thrombosed.
- Both kidneys appear normal in size & attenuation values. No evidence of hydronephrosis on both sides. Perinephric region is normal bilaterally. Both ureters appear normal. Few cortical cysts noted in right kidney, largest measuring 1.1×7.7 cm in the upper pole.
- Urinary bladder is moderately distended. No wall irregularity /thickening seen. No calculi / diverticuli.
- Uterus appears atrophic.
- Appendix appears normal in size and morphology. No periappendiceal fat stranding / focal collection / mass formation. No significant mesenteric lymphadenopathy.
- Small bowel – duodenum, jejunum and proximal ileum – collapsed. The distended ileal loops show no suspicious thickening / lesions.
- Cecum and IC junction – no gross thickening / lesions seen.
- Ascending, transverse, descending, sigmoid colon and rectum – moderately distended - no obvious wall thickening seen. Few diverticuli noted in cecum and sigmoid colon. Few prominent epiploic appendages with adjacent fat stranding are seen along the sigmoid colon, suggestive of epiploic

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appendagitis.

- *Atherosclerotic vessel wall calcification noted in aorta and its branches. Aorta and I.V.C. appear normal.*
- No ascites / free air in the peritoneal cavity.
- **Few peripherally enhancing soft tissue density lesions noted in the peritoneum (right lower quadrant of abdomen and rectouterine pouch), largest measuring 1.5 x 0.8 cm.**
- Abdominal wall appears normal.
- Soft tissue/subcutaneous planes are normal.
- *Visualized vertebra shows degenerative changes. No suspicious lytic / sclerotic lesions seen.*
- *Visualized thoracic viscera shows: Subsegmental atelectasis noted in the posterior basal segment of bilateral lower lobes. Focal posterior diaphragmatic eventration noted bilaterally.*

IMPRESSION:

- **Imaging features are suggestive of locally advanced carcinoma of the pancreatic tail with hepatic and peritoneal metastases, corresponding to Stage IV disease.**


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